

DERMATOLOGY SUBJECT WISE TEST DATED ON 9th MAY 2026

QUESTIONS

Q1. A 32-year-old male farm laborer presents with a single, large, light-colored patch on his left forearm that has been gradually increasing in size over the past year. He reports a diminished ability to feel touch and temperature over the patch. On examination, a 7x5 cm, well-demarcated, hypopigmented, and slightly erythematous plaque with a raised outer border is noted. Sensation to fine touch, pinprick, and temperature is significantly reduced over the lesion. The left ulnar nerve is palpably thickened and non-tender. A slit-skin smear from the lesion is negative for acid-fast bacilli. Based on the Ridley-Jopling classification, what is the most likely diagnosis?

- a) Tuberculoid (TT) Leprosy
- b) Borderline Tuberculoid (BT) Leprosy
- c) Mid-Borderline (BB) Leprosy
- d) Lepromatous (LL) Leprosy

Q2. A 45-year-old male farmer presents with a 2-year history of progressive, non-itchy skin thickening on his face and earlobes. He also reports numbness in his hands and feet. On examination, there is diffuse infiltration of the facial skin, loss of lateral eyebrows, and thickened, nodular earlobes as shown in the image. A slit-skin smear performed from an earlobe lesion reports a Bacillary Index (BI) of 5+. What is the most appropriate treatment regimen for this patient?



- a) Rifampicin 600 mg monthly + Dapsone 100 mg daily for 6 months
- b) Rifampicin 600 mg monthly + Clofazimine 300 mg monthly + Dapsone 100 mg daily + Clofazimine 50 mg daily for 12 months
- c) Rifampicin 600 mg daily + Isoniazid 300 mg daily + Pyrazinamide 1500 mg daily + Ethambutol 800 mg daily for 2 months
- d) Ofloxacin 400 mg + Minocycline 100 mg + Rifampicin 600 mg as a single dose

Q3. A 32-year-old male, diagnosed with lepromatous leprosy four months ago, has been adherent to his multi-drug therapy regimen. He presents to the dermatology OPD with a 3-day history of high-grade fever, joint pain, and the sudden eruption of multiple painful red lumps on his arms and shins, as shown in the image. On examination, he is febrile (39°C), and the lesions are tender, erythematous, subcutaneous nodules. His existing leprosy patches do not appear more inflamed. What is the most appropriate immediate management for his current condition?



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- a) Administer systemic corticosteroids
- b) Increase the dose of rifampicin and dapsone
- c) Immediately stop all anti-leprosy medications
- d) Start thalidomide as the first-line agent

Q4. A patient develops a sharply demarcated erythematous patch on his lip after taking medication. The lesion heals with hyperpigmentation and recurs at the same site upon re-exposure.



What is the diagnosis?

- a) Stevens-Johnson syndrome
- b) Fixed drug eruption
- c) Urticaria
- d) Erythema multiforme

Q5. A 45-year-old male farm laborer from a rural district in Bihar presents with a progressive inability to use his right hand for daily tasks over the past year. On examination, the hand deformity shown in the image is noted. There is a significant loss of sensation over the little finger and the medial half of the ring finger. Further examination reveals a palpably thickened ulnar nerve at the elbow and several large, hypopigmented, anesthetic patches on his back. What is the most likely diagnosis?



- a) Hansen's disease
- b) Diabetic neuropathy
- c) Syringomyelia
- d) Pancoast tumor

Q6. A 24-year-old wrestler presents with an itchy, circular rash on his arm that has been slowly expanding over the past two weeks. He mentions that some of his teammates have similar rashes. On examination, the lesion appears as shown. Which of the following is the most appropriate initial investigation to confirm the suspected diagnosis?

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| a) Wood's lamp examination | c) Skin biopsy for histopathology |
| b) KOH mount of skin scrapings | d) Tzanck smear |

Q7. A 45-year-old obese man with poorly controlled type 2 diabetes mellitus presents with a severely itchy rash in his groin for the past two weeks. On examination, there are well-demarcated, beefy red, moist plaques in the inguinal folds. Several small, erythematous satellite papules and pustules are noted at the periphery of the main plaques. A skin scraping is obtained and a potassium hydroxide (KOH) mount is prepared, the microscopic findings of which are shown in the image. What is the most likely causative organism?



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| a) Trichophyton rubrum | c) Malassezia furfur |
| b) Candida albicans | d) Microsporum canis |

Q8. A 45-year-old obese male with type 2 diabetes mellitus presents with a persistent, non-pruritic, reddish-brown patch in his right axilla that has been slowly expanding over several months. On examination, the lesion is well-demarcated with fine scaling, as shown in the image. A Wood's lamp examination is performed to aid in diagnosis. What color of fluorescence is most likely to be observed?



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| a) Bright yellow-green |
| b) Coral red |

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- c) Bluish-white accentuation
- d) Faint coppery-orange

Q9. A 35-year-old man presents with an itchy lesion on the abdomen that began as an annular plaque. After using a potent topical steroid, the lesion enlarged, lost its central clearing, and became ill-defined with reduced scaling. What is the most likely diagnosis?

- a) Tinea incognito
- b) Psoriasis vulgaris
- c) Nummular eczema
- d) Discoid lupus erythematosus

Q10. A 28-year-old female presents with an evolving rash on her left lower leg for the past 6 weeks. She initially noticed an itchy, ring-shaped patch and applied an over-the-counter cream containing clobetasol and miconazole. While the itching subsided initially, the lesion has since expanded and developed multiple tender bumps. Examination reveals an erythematous plaque with numerous perifollicular papules and pustules, as shown in the image. What is the most appropriate next step in management?



- a) Initiate oral terbinafine therapy
- b) Switch to a high-potency topical antifungal like luliconazole cream
- c) Perform incision and drainage of the largest nodules
- d) Administer intralesional triamcinolone injections

Q11. A 24-year-old sexually active male presents with a single, painless ulcer on the shaft of his penis that he first noticed 10 days ago. He reports inconsistent condom use. On examination, a 1.5 cm, well-demarcated, indurated ulcer with a clean, non-purulent base and raised, firm borders is noted. Regional inguinal lymph nodes are enlarged, firm, and non-tender. If this lesion is left untreated, what is the most likely natural course of the ulcer itself?



- a) It will spontaneously resolve without scarring within 3-6 weeks.
- b) It will become progressively painful and develop a suppurative base.
- c) It will persist and enlarge until specific antibiotic therapy is administered.
- d) It will evolve into a soft, beefy-red, granulomatous mass.

Q12. A 28-year-old male presents with a 2-week history of a non-pruritic rash on his palms and soles, as shown in the image. He also reports intermittent low-grade fever and generalized body aches. On further questioning, he recalls



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having a single, painless ulcer on his penis about a month ago, which healed on its own without any treatment. On examination, he has generalized, non-tender lymphadenopathy. Which of the following is the most specific test to confirm the diagnosis?



- a) VDRL test
- b) FTA-ABS test
- c) Dark-field microscopy of a skin lesion
- d) Gram stain and culture of a skin scraping

Q13. A 38-year-old man undergoes a mandatory health screening for an overseas employment visa. He is completely asymptomatic and denies any history of genital ulcers, rashes, or high-risk sexual behavior. His routine serological tests reveal a positive Venereal Disease Research Laboratory (VDRL) test with a titer of 1:16. What is the most appropriate next investigation to confirm the diagnosis of syphilis?

- a) Repeat VDRL test after 4 weeks
- b) Administer a single dose of intramuscular Benzathine Penicillin G
- c) Treponema pallidum Haemagglutination Assay (TPHA)
- d) Dark-field microscopy of a mucosal swab

Q14. A 28-year-old male presents with a two-week history of a non-pruritic rash, low-grade fever, and malaise. Serological tests are positive for syphilis. He is administered a single intramuscular dose of benzathine penicillin G. Six hours later, he develops high-grade fever (39.5°C), chills, myalgia, and a noticeable exacerbation of his skin rash. Which of the following best describes the underlying mechanism of this reaction?

- a) IgE-mediated degranulation of mast cells and basophils
- b) Massive release of treponemal lipoproteins and inflammatory cytokines
- c) Deposition of drug-antibody immune complexes in small vessels
- d) T-cell mediated inflammatory response to a penicillin hapten

Q15. A 10-year-old boy is brought to the dermatology clinic for evaluation of abnormally shaped permanent teeth. His medical history is significant for being born to a mother who received inadequate antenatal care. On examination, his upper central incisors are widely spaced, smaller than normal, and have a central notch, as shown in the image. Which of the following additional findings is most likely to be present in this patient as part of the same syndrome?

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| a) Interstitial keratitis | c) Chorioretinitis |
| b) Cataracts | d) Bullous myringitis |

Q16. A 38-year-old male presents with a 6-month history of well-demarcated, itchy, red patches on his elbows, knees, and scalp. He notes that the patches are covered with a silvery, flaky scale that bleeds easily if scratched. The clinical appearance of a lesion on his elbow is shown. Which of the following clinical signs is most characteristically associated with this condition upon methodical scraping of the scale?



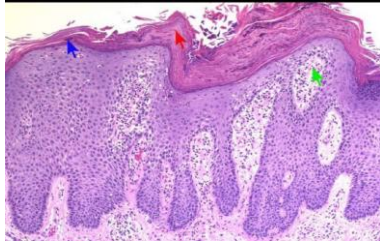
- | | |
|------------------|----------------------|
| a) Auspitz sign | c) Darier's sign |
| b) Nikolsky sign | d) Asboe-Hansen sign |

Q17. A 35-year-old male presents with itchy, well-demarcated, erythematous plaques topped with silvery-white scales on his elbows and knees for the past 6 months. On examination, gentle scraping of a scale from a plaque reveals multiple, minute, punctate bleeding points as shown in the image. What is the most likely histopathological explanation for this clinical finding?

- | |
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| a) Collections of neutrophils within the stratum corneum |
| b) Liquefactive degeneration of the basal cell layer |
| c) Dilated, tortuous capillaries in dermal papillae beneath a thinned suprapapillary plate |
| d) Intercellular edema within the epidermis leading to vesiculation |

Q18. A 35-year-old male presents with well-demarcated, erythematous plaques covered with silvery scales on his elbows and knees for the past 5 years. A skin biopsy is performed, and a key histopathological finding is shown in the image. The formation of this aggregate of inflammatory cells within the stratum corneum is primarily mediated by which of the following chemoattractants?

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- a) Leukotriene B4
- b) Histamine
- c) Transforming Growth Factor-beta
- d) Interleukin-10

Q19. A 45-year-old male complains of painful, stiff fingers and lower back, especially upon waking. Examination reveals tender, swollen distal interphalangeal (DIP) joints. He has noticed the nail changes shown in the image developing over the last year. What is the most likely underlying condition?



- a) Rheumatoid arthritis
- b) Reactive arthritis
- c) Systemic lupus erythematosus
- d) Psoriatic arthritis

Q20. A 19-year-old male presents with a sudden onset of a widespread, mildly pruritic rash over the past week. He reports having a severe sore throat and fever approximately three weeks ago, which resolved with a course of oral antibiotics. On examination, there are numerous discrete, 2-5 mm, erythematous, 'tear-drop' shaped papules with a fine, silvery scale distributed primarily over his trunk and proximal extremities, as shown in the image. Which of the following best describes the immunological mechanism underlying this patient's condition?



- a) IgE-mediated degranulation of mast cells in response to a food allergen
- b) T-cell cross-reactivity between streptococcal M protein and cutaneous keratin antigens
- c) Autoantibody formation against desmoglein proteins in the epidermal intercellular junctions
- d) Type IV hypersensitivity reaction to a hapten penetrating the stratum corneum

Q21. A 40-year-old obese male presents with dark, velvety thickening of the skin over the neck and axilla. Which of the following is the most common underlying association?

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| a) Hypothyroidism | c) Addison's disease |
| b) Insulin resistance | d) Vitamin B12 deficiency |

Q22. A 45-year-old male presents with intensely itchy, flat-topped, purple-colored lesions on his flexor wrists and ankles for the past 3 months. He reports that new lesions appear at sites of minor scratches. A close-up clinical photograph of a representative lesion is shown. What is the most likely diagnosis?



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|-----------------------|-----------------------------------|
| a) Psoriasis vulgaris | c) Discoid lupus erythematosus |
| b) Lichen planus | d) Lichen sclerosus et atrophicus |

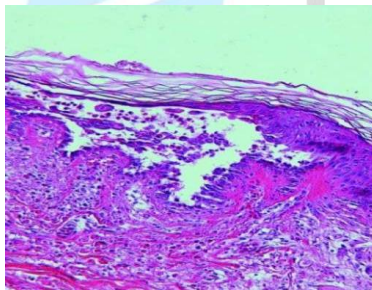
Q23. A 45-year-old woman is found to have an asymptomatic, bilateral lesion on her buccal mucosa during a routine dental examination, as shown in the image. She has no history of tobacco or alcohol use. On further questioning, she mentions having several purplish, itchy, flat-topped papules on her wrists. Which of the following histopathological findings is most characteristically associated with this condition?

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- a) Epithelial hyperplasia with varying degrees of dysplasia and hyperkeratosis
- b) Dense, band-like lymphohistiocytic infiltrate at the dermo-epidermal junction
- c) Suprabasal acantholysis with intraepidermal bulla formation
- d) Pseudomembranes containing fungal hyphae and desquamated epithelial cells

Q24. A 58-year-old male presents with a 3-month history of painful, easily ruptured blisters on his trunk and scalp. He also reports painful sores in his mouth that make eating difficult. On examination, flaccid bullae are noted on an erythematous base. Gentle pressure on the adjacent normal-appearing skin causes the epidermis to shear off (positive Nikolsky sign). A skin biopsy is performed, and the histopathology is shown. What is the most likely diagnosis?



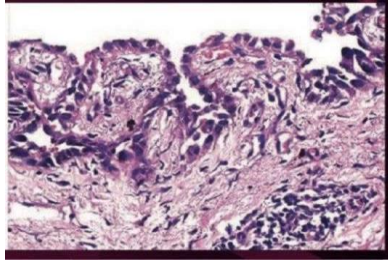
- a) Pemphigus vulgaris
- b) Bullous pemphigoid
- c) Dermatitis herpetiformis
- d) Staphylococcal Scalded Skin Syndrome

Q25. A 55-year-old man presents with painful oral ulcers followed by fragile blisters on the trunk that rupture easily, leaving erosions. Gentle lateral pressure causes the epidermis to shear off (positive Nikolsky sign). This finding is due to autoantibodies against which of the following?

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|-------------------------------|----------------------|
| a) BP180 (Type XVII collagen) | c) Desmoglein 3 |
| b) Desmoglein 1 | d) Type VII collagen |

Q26. A 52-year-old woman presents with painful oral erosions and flaccid bullae on the trunk. Nikolsky sign is positive. Skin biopsy shown .

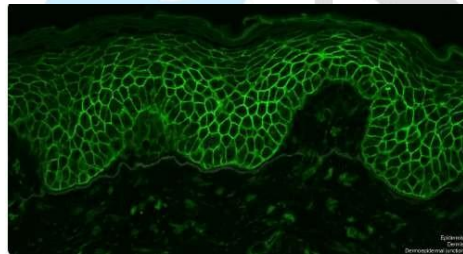
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Which of the following histopathological findings is most characteristic?

- a) Subepidermal split with eosinophils
- b) Neutrophilic microabscesses at dermal papillae
- c) Suprabasal acantholysis with “row of tombstones”
- d) Full-thickness epidermal necrosis

Q27. A 45-year-old woman presents with a 3-month history of painful erosions in her mouth, making it difficult to eat. Over the last month, she has developed a few scattered, easily ruptured blisters on her chest and back. On examination, applying gentle pressure to the normal-appearing skin results in the separation of the epidermis. A skin biopsy for direct immunofluorescence (DIF) reveals the pattern shown in the image. Which of the following proteins is the primary target of the autoantibodies in this condition?



- a) Desmoglein 3
- b) BP180 (Type XVII collagen)
- c) Tissue transglutaminase
- d) Type VII collagen

Q28. An 82-year-old man presents with a 3-week history of intensely pruritic, large, fluid-filled blisters on his trunk and flexural areas. On examination, multiple large, tense bullae are noted on an erythematous and urticarial base, as shown in the image. The blisters are difficult to rupture, and gentle pressure on the perilesional skin does not induce new blister formation (negative Nikolsky sign). A skin biopsy for direct immunofluorescence is performed. Which of the following findings would be most consistent with the diagnosis?



- a) Intercellular 'fish-net' pattern of IgG and C3 deposition within the epidermis
- b) Linear deposition of C3 and IgG along the basement membrane zone
- c) Granular deposition of IgA in the dermal papillae



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- d) Deposition of IgG against Type VII collagen at the sublamina densa

Q29. A 65-year-old male presents with a 3-week history of an intensely pruritic blistering eruption on his trunk and extremities, as shown in the image. Histopathology of a blister edge reveals a subepidermal split with a predominantly neutrophilic infiltrate. Direct immunofluorescence (DIF) of perilesional skin demonstrates a homogenous linear deposition of IgA along the basement membrane zone. What is the most likely diagnosis?



- a) Bullous pemphigoid
b) Linear IgA bullous dermatosis
c) Dermatitis herpetiformis
d) Epidermolysis bullosa acquisita

Q30. A 35-year-old male presents with a 6-month history of an intensely pruritic rash. The itching is so severe that it often disrupts his sleep. On examination, you note symmetrical, grouped, excoriated papules and vesicles on an erythematous base, predominantly over his elbows, knees, and buttocks, as shown in the image. He denies any significant gastrointestinal symptoms. In addition to a skin biopsy for histopathology and immunofluorescence, which of the following investigations is most crucial for the long-term management of this patient's underlying condition?



- a) Serum Antinuclear Antibody (ANA) levels
b) Serology for anti-tissue transglutaminase (tTG) IgA antibodies
c) Total serum IgE levels
d) Patch testing for common contact allergens

Q31. A 28-year-old male, recently diagnosed with epilepsy and started on carbamazepine 3 weeks ago, presents to the emergency department with a high-grade fever and a painful, rapidly spreading rash. On examination, he is tachycardic and has extensive dusky red macules coalescing into large, flaccid bullae. There is significant sloughing of the epidermis, estimated to involve 40% of his body surface area, with a positive Nikolsky sign. He also has severe hemorrhagic crusting of the lips and conjunctival injection. Based on the likely diagnosis, which of the following parameters is considered the most significant independent predictor of mortality?

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- a) Age > 40 years
- b) Serum bicarbonate < 20 mmol/L
- c) Percentage of body surface area detached
- d) Presence of malignancy

Q32. A 45-year-old male, recently started on allopurinol for gout, presents to the emergency department with a three-day history of high-grade fever, malaise, and a rapidly spreading painful rash. On examination, he has dusky, erythematous macules, flaccid bullae, and large sheets of epidermal detachment affecting approximately 40% of his body surface area. There is severe hemorrhagic crusting of the lips and conjunctival injection. Gentle lateral pressure on the skin causes further separation of the epidermis. A skin biopsy is obtained from the edge of a lesion. Which of the following histopathological findings is most characteristic of this condition?

- a) Intraepidermal cleavage within the granular layer with acantholysis
- b) Suprabasal intraepidermal cleavage with acantholytic cells (Tzanck cells)
- c) Subepidermal cleavage with full-thickness necrosis of the epidermis and a sparse dermal infiltrate
- d) Subepidermal cleavage with a dense dermal infiltrate rich in eosinophils

Q33. A 24-year-old woman presents to the dermatology OPD with concerns about developing white patches on her hands and around her mouth over the past year. She reports that the patches are asymptomatic and have been gradually increasing in size. On examination, well-demarcated, 'chalky' white macules are noted, as shown in the image. A Wood's lamp examination accentuates the lesions. What is the primary pathophysiological mechanism responsible for this condition?



- a) Autoimmune destruction of melanocytes
- b) Defective melanosome transfer to keratinocytes
- c) Decreased tyrosinase activity due to a genetic mutation
- d) Post-inflammatory inhibition of melanogenesis



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Q34. A 32-year-old woman, who started oral contraceptive pills 6 months ago, presents with gradually developing asymptomatic, blotchy brown patches on her cheeks and forehead, as shown in the image. The lesions become darker on sun exposure. A Wood's lamp examination is performed to determine the depth of pigmentation. Which of the following findings would suggest a more favorable prognosis and better response to topical therapies?



- a) Accentuation of pigmentary contrast between lesions and normal skin
- b) No significant change in the appearance of the lesions
- c) Coral-red fluorescence within the pigmented patches
- d) Bright blue-green fluorescence of the affected areas

Q35. A 12-year-old boy is brought to the dermatology OPD with a single, well-demarcated patch of depigmentation on the right side of his forehead that appeared 6 months ago. The lesion, as shown in the image, follows a dermatomal pattern and has not spread further in the last 3 months. His family history is negative for similar skin conditions or autoimmune diseases. Based on this presentation, which of the following statements most accurately describes the likely nature of his condition?



- a) The condition is likely to have a rapid onset and then stabilize within 1-2 years.
- b) There is a strong association with other autoimmune disorders like thyroiditis.
- c) Koebner phenomenon is a common triggering factor for the development of new lesions.
- d) The lesions will likely progress to involve contralateral and acral sites symmetrically.

Q36. A 22-year-old hostel student presents with intense itching, especially at night. On examination, multiple excoriated papules are present in the interdigital spaces, wrists, and periumbilical region. A burrow is identified.

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What is the most appropriate treatment?

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| a) | Oral terbinafine | c) | Oral acyclovir |
| b) | Topical permethrin 5% | d) | Topical mupirocin |

Q37. A 68-year-old male presents with a chief complaint of gradual darkening of his skin over the past year, which he finds cosmetically distressing. He has a past medical history of paroxysmal atrial fibrillation, for which he has been on stable medication for the last three years. On examination, there is a distinct slate-grey to bluish discoloration, most prominent on his cheeks, nose, and the dorsum of his hands, as shown in the image. The rest of his physical examination is unremarkable. Which of the following medications is the most likely cause of his skin changes?



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| a) | Amiodarone | c) | Hydroxychloroquine |
| b) | Minocycline | d) | Zidovudine |

Q38. A 55-year-old farmer presents with a pigmented lesion on his back that he says has been 'changing and growing' over the past year. He has a history of significant sun exposure. On examination, the lesion shown is noted. Based on the clinical findings, what is the most appropriate next step in management?





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- a) Shave biopsy for histological confirmation
- b) Excisional biopsy with 1-3 mm margins
- c) Reassurance and follow-up in 6 months
- d) Cryotherapy with liquid nitrogen

Q39. A 68-year-old male farmer presents with a slow-growing lesion on the side of his nose that has been present for over a year. He reports that it occasionally bleeds with minor trauma and has never fully healed. On examination, a 0.8 cm translucent, pearly papule with a rolled border and fine, overlying telangiectasias is observed. What is the most appropriate next step in managing this patient?



- a) Topical imiquimod cream
- b) Cryotherapy with liquid nitrogen
- c) Shave biopsy for histopathological confirmation
- d) Immediate Mohs micrographic surgery

Q40. A 68-year-old man presents with a painful, non-healing ulcer on his left shin that has been progressively enlarging over the past 6 months. He reports a history of a severe thermal burn at the same site 40 years ago. On examination, there is a 4x3 cm ulcer with indurated, everted edges and a foul-smelling discharge, located within a large, atrophic scar. The development of this lesion is most strongly associated with which of the following underlying processes?

- a) Chronic inflammation and scarring
- b) Cumulative ultraviolet radiation exposure
- c) Inherited nucleotide excision repair defect
- d) Iatrogenic immunosuppression

Q41. A 68-year-old farmer presents with a persistent, rough, scaly patch on his forehead that has been present for several years. He notes that it sometimes crusts over and falls off, only to reappear. On examination, the lesion shown in the image is noted, which feels like sandpaper on palpation. What is the most common malignancy this lesion may progress to?

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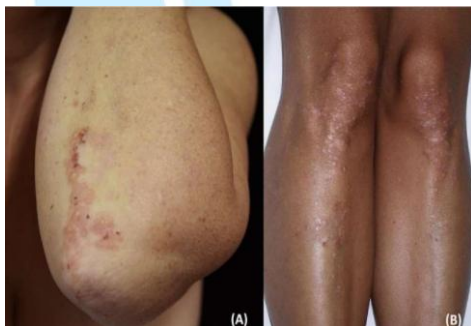


- a) Basal cell carcinoma
- b) Squamous cell carcinoma
- c) Malignant melanoma
- d) Keratoacanthoma

Q42. A 72-year-old male presents for a routine check-up. He is concerned about a 'wart' on his back that has been present for several years. On examination, the lesion . It is a well-demarcated, waxy, 1.5 cm papule with a classic 'stuck-on' appearance. Which of the following histopathological findings is most characteristic of this condition?

- a) Asymmetric proliferation of atypical melanocytes in nests
- b) Proliferation of benign basaloid keratinocytes with horn cysts
- c) Nests of basaloid cells with peripheral palisading and stromal retraction
- d) Downward proliferation of atypical keratinocytes with keratin pearls

Q43. A patient presents with intensely pruritic grouped vesicles over the elbows and knees. Which of the following is the most appropriate associated investigation?



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| a) ANA | c) Serum IgE |
| b) Anti-tTG IgA | d) Rheumatoid factor |

Q44. Identify the following lines in dermatology:





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- a) Blaschko's lines
b) Marionette Lines
c) Hinderers Lines
d) Relaxed Skin tension Lines

Q45. Match the following wood lamp findings with the disease:

1. Erythrasma	A. Pink fluorescence
2. Pityriasis versicolor	B. Coral Red fluorescence
3. Tinea capitis	C. Milky white fluorescence
4. Vitiligo	D. Blue florescence
	E. Yellow florescence

- a) 1-B, 2-E, 3-D, 4-C
b) 1-B, 2-E, 3-A, 4-D
c) 1-A, 2-C, 3-B, 4-E
d) 1-E, 2-D, 3-C, 4-B

Q46. Match the following wood lamp findings with the disease

1. Erythrasma	B. Coral Red fluorescence
2. Pityriasis versicolor	E. Yellow fluorescence
3. Tinea capitis	D. Blue florescence
4. Vitiligo	C. Milky white fluorescence

- a) 1-B, 2-E, 3-D, 4-C
b) 1-E, 2-B, 3-C, 4-D
c) 1-C, 2-D, 3-B, 4-E
d) 1-D, 2-C, 3-E, 4-B

Q47. Statement 1: Histopathological examination of a patient with Flaccid bulla and painful erosions showing suprabasal blister and inflammatory infiltrate in the blister cleft.

- a) Statement 2: Row of tombstones appearance is diagnostic of Pemphigus vulgaris.
b) Statement 1 is false and Statement 2 is true.

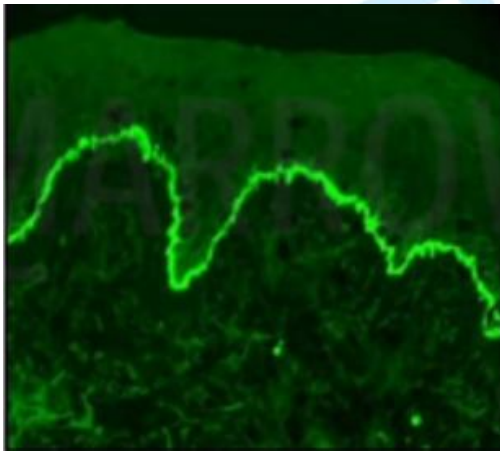


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- c) Statement 1 is true and Statement 2 is false.
- d) Both the statements are true but 2 is not the conclusion of 1
- e) Both the statements are true but 2 is the conclusion of 1.

Q48. A patient on Anti PD1 immunotherapy (Nivolumab) comes with pruritus, eczema and urticarial rashes. Histopathological examination reveals subepidermal blister and Direct Immunofluorescence of the lesions showed Linear C3 and IgG on Basement membrane. What is the diagnosis?



- a) Dermatitis Herpetiformis
- b) Bullous pemphigoid
- c) Pemphigus vulgaris
- d) Pemphigus foliaceus

Q49. Which of the following are cause(s) of non-scarring alopecia?

1. Alopecia areata
2. Telogen effluvium
3. Androgenic alopecia
4. Frontal fibrosing alopecia

- a) 1,2,3
- b) 3, 4

- c) 2,3
- d) Only 4

Q50. 24 year old young male presented with asymptomatic scaly lesions over back, symmetrical in distribution. Images of the lesions are given, Identify the disease and the pattern of lesions:



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

DERMATOLOGY SUBJECT WISE TEST DATED ON 9th MAY 2026



- a) Collarette scale, Christmas tree pattern, Pityriasis rosea
- b) Wickhams striae, Lichen planus
- c) Christmas tree pattern, Lichen planus
- d) Woronoff ring, Seborrheic dermatitis

ANSWERS

1	b
2	b
3	a
4	b
5	a
6	b
7	b
8	b
9	a
10	a
11	a
12	b
13	c

14	b
15	a
16	a
17	c
18	a
19	d
20	b
21	b
22	b
23	b
24	a
25	c
26	c

27	a
28	b
29	b
30	b
31	c
32	c
33	a
34	a
35	a
36	b
37	a
38	b
39	c

40	a
41	b
42	b
43	b
44	a
45	a
46	a
47	d
48	b
49	b
50	a